

MP-IMG-002 Knee MRI

1 Purpose and scope

This policy describes when Meridian Health Plan considers MRI of the knee medically necessary. It applies to CPT codes 73721 (without contrast), 73722 (with contrast) and 73723 (without and with contrast). Meridian Health Plan is a fictional payer created for a portfolio project, and this document is not clinical guidance.

2 Definitions

Conservative therapy for the knee means at least 4 weeks of treatment that combines two or more of the following: anti-inflammatory or analgesic medication, supervised physical therapy or a home exercise program, bracing or a walking aid, activity modification, and injection therapy. Mechanical symptoms mean locking, catching or giving way of the knee. Radiographs means at least two views of the same knee, weight-bearing when possible.

3.1 Radiograph prerequisite

Plain radiographs of the affected knee, taken within 60 days before the MRI request, are required before a knee MRI is approved. The radiograph result must be stated in the clinical note. This requirement is waived for acute trauma with suspected ligament rupture and for suspected knee dislocation.

3.2 Suspected meniscal tear

Knee MRI is approved for a suspected meniscal tear when the patient has mechanical symptoms and at least one supportive examination finding (positive McMurray test, joint line tenderness or effusion), and has not improved after at least 4 weeks of conservative therapy.

3.3 Suspected ligament injury

Knee MRI is approved without a trial of conservative therapy after acute trauma when the examination shows effusion and instability, such as a positive Lachman test, a positive anterior or posterior drawer test, or valgus or varus laxity.

3.4 Suspected occult fracture or osteonecrosis

Knee MRI is approved when radiographs are normal or nondiagnostic and pain persists or is out of proportion to the findings, for example inability to bear weight after an injury, or risk factors such as long-term corticosteroid use.

3.5 Osteoarthritis

MRI is not medically necessary to diagnose or monitor osteoarthritis that is visible on radiographs. MRI may be approved for a patient with osteoarthritis when mechanical symptoms suggest a

meniscal tear, or when surgery other than joint replacement is planned.

3.6 Suspected infection or tumor

Knee MRI is approved without a trial of conservative therapy when the note documents fever with a warm swollen joint, elevated inflammatory markers, a palpable mass, or unexplained night pain.

4 Exclusions

Knee MRI is not considered medically necessary in these situations:

- nonspecific knee pain without documented examination findings
- no physical examination of the knee documented in the clinical note
- imaging requested only for patient preference
- a repeat MRI within 12 months without a change in clinical status
- imaging of an asymptomatic knee

5 Required documentation

The request must include laterality (right or left); the date and result of radiographs; physical examination findings, including effusion, range of motion, McMurray test and Lachman test where relevant; each conservative treatment tried, with dates, duration and response; the mechanism of injury if traumatic; and the CPT and ICD-10 codes.